
THE BELFIORE FRONT DESK

The Client Experience Handbook

A working handbook for the Belfiore front desk

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- 01** What good looks like here
 - 02** Doing the job day to day
 - 03** When it is difficult
 - 04** The client who is not in the room
 - 05** How we know it is working
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PREPARED FOR

The administrative and front-of-house team, Belfiore Medical Aesthetics

FOR USE ON

25 and 26 September 2026, and every working day after that

CLASSIFICATION

Confidential to Belfiore Medical Aesthetics

PREPARED BY

Consult for Africa

BELONGS TO

How to use this handbook

SESSIONS

10

across two days, five hours each

DIFFICULT CLIENTS

6

and a different response for each

DAYS

30

of plan you will leave with

This is not a brochure and it is not a certificate. It is a working book. Write in it.

It has three parts.

- **Part one, day one.** Sessions 1 to 5. What good looks like at Belfiore, and how to do the ordinary things well.
- **Part two, day two.** Sessions 6 to 10. What to do when it is difficult, what happens after a client leaves, and how any of it is kept up.
- **Part three, the desk reference.** The scripts, the phrase bank, the escalation map, the message set and the recall calendar, gathered in one place. This is the part you keep at the desk and use on a Tuesday afternoon in December. If you only ever open one part of this book again, open that one.

Every session follows the same shape, so you always know where you are:

Marker	What it means
Why this matters	The commercial reason. What it costs Belfiore when this is done badly
What you will be able to do	The test. If you cannot do these things at the end, the session did not work
The content	The actual method, and the words to use
Say this	Wording you can use as it is written. Make it your own, but start here
Try it	The exercise we do in the room
Your notes	Ruled lines. Use them

A word on the word *client*. In a hospital the person is a patient. At Belfiore she is a client who is also, at moments, a patient. She is spending her own money on something she has chosen, she is judging the result in a mirror, and she can go elsewhere tomorrow. That is why this handbook says client, and it is why the standard here is higher than a hospital's, not lower.

The ground rules for the two days

Six of them. They are here because the room only works if everybody agrees to them.

1. **Nothing said in this room about a named client leaves it.** We will use real situations. Use first names or no names.
2. **Nobody is in trouble.** If something is broken, we want to hear it. A problem that stays hidden for two days stays broken for another year.
3. **Say the hard thing.** "I do not know what to say when that happens" is the single most useful sentence anybody can offer in the next two days.
4. **Phones down, except when we are using them.** We will use your actual WhatsApp and Instagram threads in two sessions. The rest of the time, down.
5. **You will be asked to practise out loud.** It is uncomfortable for about four minutes and then it is not. Reading a script silently teaches you nothing.
6. **Everything you write in this book is yours.** We do not collect it. Only the thirty-day plan at the end is shared, and it is shared with Dr Rapu because she is the one who has to support it.

Before we start: where you are today

Answer honestly. Nobody sees this but you, and you will answer it again at the end of day two.

	Not yet	Sometimes	Usually	Always
I know exactly what Belfiore expects of me when a client makes contact				
I can quote a price without checking with somebody				
I can answer most treatment questions a client asks				
I am confident with a client who is angry				
I know what to say when somebody is unhappy with her result				
I know who to escalate to, and how fast				
I know which clients are due back and when				
I know what I am allowed to decide on my own				

The one thing I most want to be able to do by the end of day two

PART ONE. DAY ONE

01 Where the money is: the Belfiore client journey

Why this matters. Most clinics put their effort into the beginning of the journey, where the marketing is, and almost none into the middle and the end, where the money actually is. A client who is won and then lost at month four costs more than a client who was never won, because you paid to acquire her and then gave her away.

What you will be able to do. Describe the Belfiore journey end to end. Name the moments where a client is most likely to be lost. Say what your own role does at each one.

The journey, in eleven moments

#	The moment	What is being decided	Owner
1	She sees us	Whether we look like somewhere she would trust with her face	Marketing
2	She makes contact	Nothing yet. She is testing us	The desk
3	She gets a reply	Whether we are serious, and whether she is going to be looked after	The desk
4	She asks the price	Whether the price is a barrier or a signal of quality	The desk
5	She books a consultation	She has committed time, not money. Still reversible	The desk
6	She arrives	Whether the place matches the promise	The desk
7	She waits	Whether she is a person here or a number	The desk
8	She is seen	Whether the clinician is worth the money	Clinical
9	She pays and leaves	Whether she feels she got value, and whether she has a next date	The desk
10	The days after	Whether anybody cared once the money was taken	The desk
11	The recall point	Whether she comes back or drifts to somewhere newer	The desk

Eight of the eleven moments belong to the front desk. That is the whole argument of these two days in one table.

The four moments where clients are most often lost

1. **Between contact and reply.** The gap. She has messaged three clinics. The order the replies arrive in matters more than almost anything else we do.
2. **At the price question.** Not because the price is too high, but because of how it is delivered. A price given with confidence is a different product from the same price given apologetically.
3. **In the days after treatment.** Silence after payment is the loudest message a clinic sends.
4. **At the recall point.** Nobody calls, she does not think about it, twelve weeks becomes ten months, and by then somebody else has her.

Try it

On the wall we will map Belfiore's real journey using your own enquiry threads from the last month. For each of the eleven moments you will write what actually happens today, not what is supposed to happen.

The moment in our journey I think we lose the most people, and why

02 The Belfiore service standard

Why this matters. You cannot be trained against nothing, coached against nothing or defended against nothing. When there is no written standard, every disagreement about whether something was handled well becomes a disagreement about personalities. When there is one, it becomes a simple question of fact, and that protects you as much as it protects the client.

What you will be able to do. State the response-time standard for every channel. Say what you are authorised to decide alone. Identify what must be escalated, to whom and how fast.

What a standard has to cover

A standard that only says "be friendly and professional" is not a standard, because nobody can fail it and nobody can be held to it. A real one is specific enough to be checked. Ten areas.

Area	The question the standard must answer
First response, opening hours	How fast, on each channel, with no exceptions
First response, out of hours	What goes out immediately, and when the real reply comes
Ownership	Who is responsible for an enquiry until it is closed
Outcome	What must be recorded before an enquiry can be considered finished
Price	Whether it is given, deflected or quoted only in consultation
Waiting	The longest a client waits before somebody speaks to her
Privacy	What may be said, and where
Complaints	How quickly they are logged, and how quickly a result complaint reaches a clinician
Aftercare	Who contacts a treated client, and within what time
Next appointment	What must happen before a client walks out of the door

Draft it here

We will fill this in together in the room. What you write here is what goes to Dr Rapu for sign-off, so write what you can actually deliver on a busy Saturday, not what sounds impressive on a Tuesday.

Area	Our standard
Instagram and WhatsApp, opening hours	
Missed calls	
Email	
Out of hours	
Who owns an enquiry	
What is recorded before an enquiry closes	
Giving a price	
Waiting	
Logging a complaint	
Aftercare contact	
Next appointment	

What you can decide, and what you cannot

The second half of a standard is authority. A team with no authority escalates everything, which is slow and expensive, or invents its own rules, which is worse. Write in what is agreed in the room.

Situation	I can decide	I must ask
Moving an appointment		
Waiving a late-cancellation charge		
Giving a discount		
Offering a complimentary review		
Answering a clinical question		
Confirming whether someone is a client		
Refunding a deposit		
Replying to a public comment		

A boundary you can name is a boundary you can hold. "That is not something I can decide, but I can get you an answer by four o'clock" is a strong sentence. "I am not sure, let me see" is not.

The decision I most often have to make and am least sure I am allowed to make

03 First contact on every channel

Why this matters. This is the highest-value fifteen minutes in the business. She has messaged you and two other clinics. Nothing about your treatments, your clinician or your equipment is visible to her yet. All she has is how you answered.

What you will be able to do. Run a first contact on any channel through to a booked consultation or a recorded outcome. Answer a price question with confidence. Handle three or four enquiries at once without losing one.

The four channels are not the same channel

Channel	What she expects	The trap
Instagram DM	A reply in minutes, informal, often late at night	Answering like a form letter. She is here because the page felt personal
WhatsApp	Fast, conversational, voice notes welcome	Letting a thread go quiet mid-conversation. On WhatsApp, silence reads as rejection
Phone	A person, immediately, who can actually help	Putting her on hold to "check", then not coming back
Walk-in	To be acknowledged within seconds, even if she then waits	Finishing what you were doing first. Ten seconds of eye contact buys you five minutes

Whatever the channel, the clock starts the moment she sends it, not the moment you see it.

The anatomy of a first reply

Five parts, in this order. It takes about thirty seconds to write and it works on every channel.

- 1. Greet her by name** if you have it. If not, ask for it in the same message.
- 2. Name yourself.** She should be dealing with a person, not an account.
- 3. Answer what she actually asked**, first, before anything else. Not a brochure. Not a redirect.
- 4. Give one piece of useful information** she did not ask for but will need.
- 5. Offer the next step with two specific options**, never an open question.

Good afternoon, thank you for reaching out to Belfiore. My name is _____.

Yes, we do offer that, and the price starts at N_____ depending on how much is needed, which the doctor will confirm at your consultation.

Most people find the consultation itself is the useful part, because you get an honest view of whether it is the right treatment for what you actually want.

Would Thursday at 11am or Saturday at 1pm work better for you?

The last line is the one people get wrong. "Let me know when you would like to come in" puts the work on her, and she is busy. Two specific times ask for a yes or a no, and either answer moves you forward.

The price question

This is the moment most enquiries are lost, and it is almost never the number that loses them.

Four moves, in order.

1. **Give the price.** Do not deflect it, do not say "it depends", do not say you will check and revert. Deflection reads as either hiding something or not knowing your own business.
2. **Attach it to what she gets.** A number on its own is an expense. A number attached to a result is an investment.
3. **Name the variable honestly** if there is one, and say who decides it.
4. **Move straight on** to the next step, in the same message. Do not leave the price sitting alone at the end of a message. It will be the last thing she reads.

The treatment is from N_____, and that covers the consultation, the treatment itself and your two-week review.

How much you need depends on the area, and the doctor will tell you exactly at the consultation rather than guessing now.

Shall I hold Thursday at 11am for you?

When she asks for a discount

She is not usually asking for money. She is asking to be reassured that she is not being overcharged, or she is testing whether the price is real. A discount conceded at the desk teaches her that all your prices are negotiable, and she will tell her friends.

Our prices are the same for everybody, and I would not be able to change them.

What I can tell you is what is included, because people are sometimes comparing a price here with a price elsewhere that does not include the review or the aftercare.

Would it help if I sent you exactly what your treatment covers?

Three things you may offer instead of a discount, if the standard allows it. Confirm which in session 2.

- A consultation at a time that suits her
- A treatment plan staged over two visits so the cost is spread
- The honest answer that a cheaper treatment may suit her better

The five things worth knowing before you book her

You are not interrogating her. You are making sure she arrives at the right appointment, with the right clinician, with realistic expectations. Two or three of these, woven into the conversation.

1. What is she actually hoping to change? Her words, not a treatment name.
2. Has she had anything done before, here or elsewhere?
3. Is there an event or a date she is working towards?
4. Has she seen something online she is comparing us to?
5. Is there anything the doctor should know about her health?

Question 3 is the most commercially useful question on this list, and almost nobody asks it. A wedding in eight weeks changes the treatment plan, the urgency and the value of the appointment to her.

When you cannot answer

Never guess, and never invent. But "I do not know" on its own loses her. Pair it with a commitment.

That is a question for the doctor rather than me, and I would rather get you the right answer than guess.

Let me put it to her and come back to you before _____ today.

Then come back. Even if the answer is not ready. Especially if the answer is not ready.

Try it

We will take six real threads from your own Instagram and WhatsApp, cover the replies, and rewrite them together. Then role play a phone enquiry, a walk-in during a busy moment, and a price objection.

The enquiry I find hardest, and the sentence I never know how to say

04 In the clinic, from the door to the till

Why this matters. She has already paid, in the sense that she has committed. What happens between the door and the till decides whether she does it again. It is also the cheapest part of the whole journey to improve, because almost all of it is free.

What you will be able to do. Run an arrival and a checkout to the standard. Tell a waiting client what is happening before she has to ask. Close a visit with the next appointment booked.

Arrival: the first fifteen seconds

Do	Why
Look up and acknowledge within five seconds, even mid-task	Being seen is most of what she wants at the door
Use her name as soon as you have it	It converts a transaction into a visit
Confirm what she is here for, quietly	It catches booking errors before the clinician does
Tell her the plan: where to sit, how long, what happens next	Uncertainty is what makes waiting feel long
Offer water or tea before she asks	Costs nothing, remembered every time

The wait

A fifteen-minute wait nobody explained feels longer than a forty-minute wait somebody managed.

The rule: no client waits more than the standard without being told why, by a person, with a new time. Not a sign. Not an apology at the end.

Mrs _____, I want to keep you updated. The doctor is running about twenty minutes behind because of the appointment before yours.

Can I get you something while you wait, and would you like me to come and find you if you would rather step out?

Notice there is no excuse in that, and no over-apology. A fact, a new expectation, and an offer.

Privacy at the desk

A medical aesthetics reception is a small room where people who know each other sit near each other. Assume every word is heard.

- Do not say a treatment name across a room.
- Do not read a file where a screen faces the seating.
- Do not take a call about one client while another is standing at the desk.

- If something must be discussed, move it: step away, take it to a room, or say you will call her back in five minutes.
- Two clients who know each other may be in the same waiting room. Never introduce them, never acknowledge that they have anything in common, and never look as though you have noticed.

The handover to the clinician

Thirty seconds that make the clinician look prepared and the client feel expected.

- Her name, what she has come for, and anything that changed since booking.
- Anything she said at the desk that matters: nervous, in a rush, unhappy about something last time.
- Anything about her history she volunteered.

Checkout and payment

The most commonly wasted ninety seconds in the practice.

Step	What good looks like
Ask how it went	An open question, before the money. It surfaces a small problem while it is still small
Take payment cleanly	Know the methods, the receipt, and what the invoice will say. Never apologise for taking money
Give aftercare in writing	Verbal aftercare is forgotten in the car park. Give it on paper or by message, always
Book the next appointment	Now, while she is standing there and pleased. This is the whole session in one line
Tell her what happens next	"I will check in with you tomorrow" sets the expectation you are about to meet

How did you find it today?

Lovely. I am going to send your aftercare to your WhatsApp so you have it in writing, and I will check in with you tomorrow to see how you are getting on.

Dr _____ suggested we see you again in _____ weeks. Shall I put you in for the _____ so you have the slot you want?

Try it

We will walk your actual reception, timed, from the door to the seat to the desk, and write down every gap we find. Then we will practise a checkout, including the next appointment, until it is natural.

What our reception does well, and the one thing I would change tomorrow

05 Confidentiality, consent and discretion

Why this matters. This is the part where a mistake cannot be recovered with good service. One confirmed attendance, one shared photograph, one overheard name, and a client is gone permanently and talking. It is also law: the Nigeria Data Protection Act 2023 applies to Belfiore, and it applies to you personally in how you handle records.

What you will be able to do. State what may and may not be disclosed, and to whom. Take and record photographic consent correctly. Handle a request to confirm whether somebody is a client. Keep a front-desk conversation private in a small reception.

The duty, in plain language

You hold information about people that they have not told their own families. The law gives you four obligations that matter at the desk.

- 1. Only collect what you actually need.** A phone number and what she came for, not her life story.
- 2. Only use it for what she gave it for.** A number given to confirm an appointment is not a marketing list.
- 3. Only share it with people who need it to do their job.** Inside Belfiore as much as outside.
- 4. Keep it safe, and know who has seen it.** Locked, password-protected, not on a personal phone, not in a group chat.

The test that covers almost everything: *would this client be surprised, or embarrassed, if she knew I had done this?* If yes, do not do it.

The seven rules at the desk

- **Never confirm that a named person is a client.** Not to a friend, not to a relative, not to a spouse, not to somebody who says they are paying the bill, not to somebody who is already standing in the waiting room.
- **Never discuss one client in front of another,** including using a treatment name loudly.
- **Never put client information in a personal WhatsApp,** including a group with colleagues in it.
- **Never take a photograph of a client, a screen or a file on a personal phone.**
- **Never forward a client photograph to anybody,** including another clinician, without checking who is allowed to receive it.
- **Never leave a screen, a diary or a form facing the waiting area.**
- **Never assume seniority equals need to know.** Somebody being senior does not mean they need this client's file.

The call you will get

Somebody rings and asks whether a named person has an appointment. The answer is the same whoever is asking, and the skill is saying it without sounding as though you are hiding something.

I am not able to confirm whether anybody is a client here, or share anything about who does or does not come to us. It is the same answer for everybody, and it is the reason our clients trust us.

If you would like, I can take your number, and if there is somebody here who wants to call you back, they will.

Say it warmly. The warmth is what stops it sounding like an accusation. And note the second half: you have offered to help without confirming anything at all.

Photographs: two different permissions

This is the one people get wrong most often, and it is the one that does the most damage.

	The clinical record photograph	The marketing photograph
What it is for	Her file, to compare before and after	Instagram, the website, a brochure
Is consent needed?	Yes, and it is part of her treatment consent	Yes, and it is a completely separate written consent
Can one cover the other?	No. Never. A record photograph may never be posted	No. Marketing consent does not extend to new uses
Can she change her mind?	She can ask for access and for it to be corrected	Yes, at any time, and it must come down
Where does it live?	The clinical record, secured	Only where she agreed it could be used
Who may take it?	Whoever is agreed clinically	The same, with the marketing consent in place

Three things to check before any photograph is used publicly.

- Is there a signed marketing consent for this specific client?
- Does it cover this specific use: this platform, this campaign, this crop?
- Has she withdrawn it since? Check, do not assume.

A face is identifiable even when cropped. Treat a "just the lower face" photograph as fully identifiable, because her friends will recognise her.

Eight situations. What do you do?

We will work these in pairs and then argue them out. Write your answer before we discuss it.

1. A well-known client's driver asks you to confirm her appointment time so he can plan his day.
2. A colleague asks you to send her a client's before photograph "just to show the doctor".
3. Two clients in the waiting room clearly know each other and one asks you what the other is here for.
4. A client asks you to delete a photograph she agreed to last year that is now on Instagram.
5. A journalist calls asking whether a named person is a client.
6. A client sends you a photograph of her results by WhatsApp and says "you can use this".
7. You need to discuss a client's outstanding balance and she is standing at the desk with a friend.
8. A relative calls to say a client is unwell and asks what treatment she had last week.

My answers

The rule I am least sure about, and want confirmed before day two ends

PART TWO. DAY TWO

06 The six difficult clients

Why this matters. A difficult client is not one thing, and treating her as one thing is why most service training stops working exactly here. Six different things drive difficult behaviour, and the response that works for one makes another worse.

What you will be able to do. Identify which of the six is in front of you within the first minute. Select the matching response. Hold a boundary without losing warmth. Recover yourself afterwards.

The one question

Before you respond to anybody, ask yourself silently: **what is she actually afraid of, or angry about, or short of?** Not what she is saying. What is underneath it. The answer tells you which of the six you have.

The rule that applies to all six

Acknowledge before you explain. Every single time. An explanation offered before an acknowledgement is heard as an excuse, and it escalates. Acknowledgement is not agreement, and it is not an admission of fault. It is simply showing that you have heard.

Instead of	Say
"The doctor is running late because the appointment before ran over"	"You have been waiting a long time and that is frustrating. Let me tell you where we are"
"That is our policy"	"I understand why that feels unfair. Let me explain how it works and what I can do"
"You did sign the form"	"I can see you did not expect this. Let me go through it with you"

01. The anxious one

What you hear. A lot of questions. The same question twice. "Are you sure?" "Has anybody had a problem?" Long pauses. Sometimes she goes quiet and then cancels.

What is driving it. Fear. She is about to let somebody change her face and she cannot undo it.

Your first sentence. *"It is completely normal to feel unsure about this, and I would rather you asked me ten questions than one."*

What to do. Slow down and match her pace. Give information, not reassurance. Offer the consultation as the low-risk step it actually is. Tell her she can change her mind.

Never. Rush her, or be efficient at her. Being handled quickly reads as being dismissed. Also never say "you will love it" about a result you cannot promise.

How to close. *"Nothing is decided at the consultation. You come in, the doctor tells you honestly what is possible, and you go home and think about it."*

02. The angry one

What you hear. Volume. A specific accusation. "I have been waiting forty minutes." "Nobody called me back." "I was charged twice."

What is driving it. A specific failure, usually ours, and often a small one that has been ignored.

Your first sentence. *"You are right, and I am sorry. That should not have happened."* If you do not yet know she is right: *"I can hear this has gone wrong and I want to sort it out. Tell me what happened."*

What to do. Let her finish without interrupting. Acknowledge the specific thing. Say what you will do and by when. Then do it, and come back even if there is nothing new to say.

Never. Explain first. Defend a colleague. Say "calm down". Match her volume.

How to close. *"Here is what I am going to do, and I will come back to you by _____ whether or not I have the full answer."*

03. The disappointed one

What you hear. Quiet, often. "I do not think it has worked." "It looks uneven." "This is not what I expected." Sometimes with a photograph.

What is driving it. A gap between what she expected and what she sees in the mirror. It may be clinical, it may be timing, it may be an expectation that was never corrected.

Your first sentence. *"Thank you for telling me. I want the doctor to look at this properly, and I am going to arrange that now."*

What to do. Take it seriously and move it to the clinician fast. Record exactly what she said in her words. See session 7, which is this situation in full.

Never. Assess the result. Reassure her that it looks fine. Say "it settles down" unless a clinician has told you that and told you to say it. Any of those destroys the clinician's position before the clinician has even seen her.

How to close. *"Dr _____ will see you on _____. In the meantime, if anything changes, call me directly on this number."*

04. The entitled one

What you hear. "Do you know who I am?" "I do not queue." "Just squeeze me in." Often charming rather than rude, which is harder.

What is driving it. Status. She expects to be an exception, and often she is treated as one elsewhere.

Your first sentence. *"It is really good to see you, and I want to sort this out for you."*

What to do. Recognise her genuinely, then hold the boundary warmly and immovably. Give her something real that is within the standard: the best available slot, a call when a cancellation comes up, your direct line.

Never. Bend the rule. The exception becomes the new baseline, she will expect it every time, and your colleagues will carry it. Also never get cold. Coldness is what turns this one into a complaint.

How to close. *"What I can do is put you first on the list if anything opens up, and I will call you myself."*

05. The negotiator

What you hear. "What is your best price?" "My friend paid less." "Can you do something for me?"

What is driving it. Money, but also a wish to be reassured she is not being overcharged.

Your first sentence. *"Our prices are the same for everybody, and I would not be able to change them."*

What to do. Restate value, not price. Explain what is included that a cheaper quote elsewhere may not include. Offer staging if the standard allows it.

Never. Concede at the desk. Apologise for the price. Criticise the cheaper clinic, which only makes you sound worried.

How to close. *"Let me send you exactly what your treatment includes, so you are comparing the same thing."*

06. The public one

What you hear. "I am going to post about this." Or you find out she already has.

What is driving it. An audience. She feels she has to be loud to be heard, usually because something quieter was ignored first.

Your first sentence, in public. *"I am so sorry this has been your experience. I am _____ and I am going to look into this personally. Please send me a message so I can get your details."*

What to do. Reply once, publicly, fast, warmly, with a name and an offer to move it private. Then actually move fast. Escalate the same day. A public complaint answered within an hour reads as a good clinic; the same complaint answered on Thursday reads as a bad one.

Never. Argue publicly. Delete the comment or block her, which turns one angry person into a story. Go silent. Or say anything at all about her treatment in public, which breaches her confidentiality even though she raised it.

How to close, in public. *"Thank you for letting me take this offline, I will come back to you today."* Nothing else. No details, ever.

The phrase bank

Situation	Say this	Not this
She is upset	"That should not have happened"	"Unfortunately our policy is"
You do not know	"I would rather get you the right answer than guess"	"I think so"
You need time	"Let me look into it and come back to you by four"	"I will get back to you"
She wants a discount	"Our prices are the same for everybody"	"Let me see what I can do"
She asks a clinical question	"That is one for the doctor, and I will get it to her today"	"Usually it is fine"
She is waiting	"You have been waiting a while. Here is where we are"	"It should not be long now"
She threatens to post	"I would like to sort this out for you properly"	"That will not be necessary"
She is nervous	"I would rather you asked me ten questions than one"	"Everybody says that"

Recovering yourself

A difficult interaction costs you something and pretending otherwise is how people burn out on a front desk.

- Take two minutes before the next client. Two minutes, not two seconds.
- Tell one colleague what happened. Not to complain, to put it down.
- Write it in the log below while it is fresh, then close the book on it.
- Separate the behaviour from you. She was not angry with you. She was angry.

My difficult client log

Date	What happened	Which of the six	What I said	What I would do differently

Try it

Six role plays, one for each type, using situations from your own baseline survey answers. You will play both sides, which is the part that actually teaches.

07 The dissatisfied result

Why this matters. This is the situation that is specific to your business and it is the one that does the most damage when it is handled badly. It is emotional, it is often clinical, and you are the first person to meet it. Handled well, a dissatisfied client frequently becomes the most loyal client in the book, because you proved what you do when things go wrong.

What you will be able to do. Take a result complaint without diagnosing, defending or conceding. Escalate it to the right clinician within the standard. Record it so it cannot be lost. Manage the hours in between.

The rule

You never assess a clinical outcome, and you never concede one.

Not "it looks fine to me". Not "that is normal". Not "it will settle". Not "I can see what you mean, that does look uneven". The first three are clinical opinions you are not qualified to give. The fourth is an admission that binds Belfiore before a clinician has even looked.

What you do instead is the four steps.

The four steps: Hear, Hold, Hand over, Record

1. Hear. Let her say the whole thing. Do not interrupt, do not start solving. Ask one open question at the end: "When did you first notice it?"

2. Hold. Acknowledge without assessing.

Thank you for telling me, and I am sorry you are worried. That matters and we are going to look at it properly.

I am not the right person to tell you what is going on, and I would rather not guess. What I am going to do is get you in front of Dr _____.

3. Hand over. Escalate within the standard, name the person and the time, and tell her what you have done.

I have spoken to Dr _____ and she can see you on _____ at _____.

In the meantime, if anything changes or you are worried, call me directly on this number and I will pick up.

4. Record. The same day, in the system, in her words.

What goes in the record

- The date and time she raised it, and the channel she used.
- **Her words, quoted.** Not your summary. "It looks uneven on the left side" is evidence. "Client unhappy" is not.
- What treatment, on what date, with which clinician.
- What you said and what you offered.
- Who you escalated to, at what time, and what they said.
- The appointment made, and whether she attended.

The record is not paperwork. It is the difference between a clinic that learns from a complaint and a clinic that has the same complaint four times without noticing.

The escalation map

Complete this in the room, and then keep it at the desk. It is useless if it lives anywhere else.

Situation	Who	How	How fast
Unhappy with a result, no symptoms			
Pain, swelling or a possible complication			
Something she says is an emergency			
Complaint about a member of staff			
A billing dispute			
A public post or review			
A request for a refund			
A lawyer or a regulator makes contact			

The hours in between

This is where clients go public, and it is almost always because the silence felt like being ignored.

- Give her a named person and a real number.
- Tell her exactly when she will hear from you, and make it soon.
- Contact her at that time even if there is no news. *"I said I would come back to you at four. I do not have the full answer yet, but I have not forgotten you."*
- Do not let a weekend pass without contact.

When it is already public

- Reply once, publicly, within the hour if you can. Warm, named, no detail.
- Move it to private immediately.
- Escalate the same day, whatever else is happening.
- Never mention her treatment, her name or her history in public, even if she has.
- Once it is resolved, ask her, once, whether she would be willing to say so. Do not push it.

Try it

Three escalations, run live: a client unhappy with an injectable result, a client with swelling calling on a Saturday, and a review posted overnight.

The result complaint I would least like to receive, and what I would say first

08 Follow-up and aftercare contact

Why this matters. Silence after payment is the loudest message a clinic sends. It costs nothing to send a message the next day, and it is the single most commented-on thing in client feedback anywhere in this sector. It also catches complications and complaints while they are still small.

What you will be able to do. Run the aftercare contact sequence. Write a check-in message that gets a reply. Ask for a review and a referral at the right moment. Recognise in a reply the early sign of a complaint.

The sequence

When	What	Why then
At checkout	Aftercare in writing, and tell her you will check in tomorrow	Sets the expectation you are about to meet
24 hours	The check-in, from a named person	Catches a complication early, and it is when she is most anxious
7 days	How are you feeling, anything you want us to look at	The point where doubt usually starts
At the result point	The review request, and only if she is happy	Asking before she can see the result is how you get a lukewarm review
At the recall interval	The recall contact. Session 9	This is the revenue

Writing a message that gets a reply

Four rules.

- 1. From a person, with a name.** Not "Belfiore Medical Aesthetics".
- 2. Short.** Two sentences. A long message reads as marketing and gets ignored.
- 3. One question, answerable in one word.** "How are you feeling?" not "Please let us know if you have any concerns or questions regarding your recent treatment."
- 4. No sell.** Nothing in the 24-hour message asks her for anything.

Hello Mrs _____, it is _____ from Belfiore. Just checking in after yesterday, how are you feeling?

Hello Mrs _____, _____ here from Belfiore. It has been a week now, how are you getting on with it? Anything you would like us to look at, just say and I will sort it.

Reading the reply

She says	What it means	What you do
"Fine thanks"	Neutral. Not a good sign or a bad one	Note it. Ask again at the result point
"Great, thank you so much"	Genuinely pleased	This is the person to ask for a review, now
"It is a bit sore/swollen still"	Possibly normal, possibly not. Not yours to judge	Escalate per the map, same day
"I am not sure it has worked"	The early form of a result complaint	Session 7. Do not reassure. Escalate
No reply at all	Unknown, and the most common	One follow-up. Then leave her, and note it

The review request

Ask once, at the right moment, from a person, and make it easy.

I am really glad you are happy with it.

If you would not mind, it genuinely helps us when people say so publicly. Here is the link, it takes a minute: _____

And thank you either way.

The referral ask

The only moment this works is right after she has told you, unprompted, that she is happy. Not before, and never in a bulk message.

That is lovely to hear.

If a friend of yours is ever thinking about it and wants to ask questions before she books, send her to me directly and I will look after her.

Try it

You will write Belfiore's actual message set in the room, in your own words, for each point in the sequence. What you write becomes the standard.

Draft my 24-hour check-in message

Draft my seven-day message

09 Prompting, recall and rebooking

Why this matters. This is the commercial engine of an aesthetics practice and it is the part most clinics never systematise. A client who is not called back is not a lost visit, she is a lost year. Recall is not marketing. It is the core operating rhythm of the business, and it belongs to this desk.

What you will be able to do. Operate the recall list. State the return interval for each treatment line. Offer the next appointment at checkout. Run a reactivation contact. Explain a deposit without apologising for it.

Prompting: getting her through the door on the day

When	What goes out	Why
On booking	Confirmation with preparation notes and the deposit position	Prevents the "nobody told me" cancellation
72 hours before	First reminder, with an easy way to move it	Late notice is better than a no-show. Give her the chance to rebook early
24 hours before	Second reminder, with a real reply channel	The one that actually prevents no-shows
Morning of	Only for first appointments and long treatments	Any more than this becomes noise and she stops reading

The 72-hour reminder is the one that earns its keep, because it is early enough for the slot to be refilled. A 24-hour reminder that produces a cancellation has saved the client's time, not yours.

Deposits

A deposit is not a distrust of the client. It is a protection of the slot, and clinics that explain it that way get no argument.

We take a deposit of N_____ to hold the appointment, and it comes straight off your treatment on the day.

If you need to move it, just give us _____ notice and it moves with you.

Never apologise for it, never present it as an obstacle, and never waive it quietly for one client and not another. Both of those cost more than the deposit.

Recall: the intervals

These intervals are clinical and Dr Rapu and the clinical team set them. Complete this table with the clinician in the room, then keep it at the desk. Nothing on this page should be guessed.

Treatment line	Recall interval	Who contacts her	What is offered
Botulinum toxin			
Dermal fillers			
Skin treatments and medical facials			
Chemical peels			
Microneedling			
Laser hair removal			
Laser skin and pigmentation			
Body treatments			
Hair			
Wellness and IV			
Post-treatment review appointments			

Rebooking at checkout

The cheapest revenue in the business. A rebooking made at the till costs nothing. The same booking chased in March costs a campaign.

Dr _____ suggested we see you again in about _____ weeks, which takes us to _____.

Shall I put you in now so you get the time you want? We can always move it if something comes up.

Two things make that work. It is specific, and it is reversible. "Shall I book you in?" is a closed question with an easy no. "Shall I put you in for the 14th so you get the time you want, we can always move it" is an easy yes.

The recall list

Three rules, and they are the whole system.

1. **Somebody is on the list the day she is treated.** Not later, not when someone remembers.
2. **The list is worked on a fixed day, every week.** A list nobody works is a list that does not exist.
3. **Nobody falls off because a person went on leave.** The list belongs to the desk, not to a person.

Who is on it	When she goes on	When she is contacted
Treated client with a recall interval	Day of treatment	At the interval
Client who did not rebook at checkout	Day of treatment	Within 7 days
Client who cancelled and did not rebook	Day of cancellation	Within 48 hours
No-show	Same day	Same day, warmly
Six months quiet	Automatically	A reactivation message
Twelve months quiet	Automatically	A call, from a named person

The no-show call

Same day, and warm. A no-show is embarrassing for her, and most people never come back because of the embarrassment rather than the appointment.

Hello Mrs _____, it is _____ from Belfiore. We had you down for this afternoon and I wanted to make sure everything is alright.

Would you like me to find you another time? No trouble at all.

Reactivation

The clients you have not seen in six or twelve months are the cheapest new business available, and almost nobody contacts them. She did not leave because she was unhappy. She left because nobody called.

Hello Mrs _____, it is _____ from Belfiore. I was going through our records and realised it has been a while since we saw you, which is entirely our fault for not being in touch.

How have you been getting on? If you would ever like to come in for a review, just say and I will look after it.

No offer, no discount, no campaign language. A person noticing. That is what works.

Try it

We will build your recall calendar with the clinical team, then draft the reactivation message and run a no-show call both ways.

How many clients do I think we have not contacted in over a year? What would it be worth?

10 Feedback, audit and making it stick

Why this matters. Everything in this handbook fades in about six weeks unless somebody is looking. Not because anybody is lazy, but because a busy desk returns to its habits. This session is the difference between two good days and a permanently better practice.

What you will be able to do. Run the weekly service huddle. Read the five numbers and say what each one means. Receive a review of your own call without defensiveness. State your own thirty-day commitments.

Asking for feedback properly

Do	Do not
Ask a small number of people, often	Send a long survey once a year
Ask soon after the visit, but after the result is visible	Ask at the till, where she will be polite
Make it anonymous where you want the truth	Attach her name and wonder why it is all five stars
Ask one open question at the end	Ask only scores, which tell you what but never why
Report it back to the team, including the bad	Collect it and let it sit in a folder

The one open question worth asking: *"What is the one thing we could have done better?"* It outperforms every scored question ever written.

Mystery shopping your own desk

Somebody the team does not know contacts Belfiore as an ordinary client would, across every channel, and writes down exactly what happened. It is uncomfortable the first time and it is the most useful single input to coaching that exists, because it is the only way to see what a stranger actually experiences.

What is measured each time: how long the reply took, whether she got a name, whether the price was given, whether a next step was offered, whether anybody followed up.

Reviewing calls and messages

This only works if it is a coaching act, never a disciplinary one. The moment a review is used to catch somebody out, the whole system dies.

How to receive one well. Listen to the whole thing before responding. Ask what you would do differently rather than defending what you did. Take one thing, not five. And remember that everybody's recorded call sounds worse to them than it does to anybody else.

The five numbers

#	The number	How to count it	What it tells you
1	Median first response time, by channel	The middle value of the gap between her message and our reply, over a week	The biggest lever on conversion we control
2	Enquiries to booked consultations	Consultations booked divided by enquiries received	Whether we have a demand problem or a conversion problem
3	Consultations to treatments	Treatments booked divided by consultations held	Whether the consultation converts, and whether expectations are honest
4	Rebook rate at checkout	Clients leaving with a next appointment, divided by clients treated	The cheapest revenue in the business
5	No-shows and late cancellations	As a share of all booked appointments	Whether prompting and deposits are working

Two watch items: complaints logged and closed within seventy-two hours, and dormant clients reactivated in the month.

The weekly huddle

Fifteen minutes. Same day, same time, standing up. It is the single mechanism that keeps everything else alive.

Minutes	Item
0 to 3	The five numbers. Up or down, no discussion yet
3 to 6	One thing that went well, named and credited
6 to 10	One thing that went wrong, and what we change. Never who is at fault
10 to 13	Anything coming: a busy day, a clinician away, a campaign landing
13 to 15	One commitment each for the week

Who owns this

A standard with no owner is a poster. Write in the names in the room.

Responsibility	Name
Owns client experience overall	
Runs the weekly huddle	
Works the recall list	
Logs and tracks complaints	
Reports the five numbers	
Approves a public reply	

My thirty-day plan

Three commitments. Specific, small enough to actually do, and checkable by somebody else. A copy goes to Dr Rapu, because she is the one who has to support it.

Commitment 1. By when. How we will know it happened

Commitment 2. By when. How we will know it happened

Commitment 3. By when. How we will know it happened

What I need from Belfiore in order to keep these

Signed: _____ Date: _____

Where you are now

The same eight questions you answered at the start. Answer them again before you leave.

	Not yet	Sometimes	Usually	Always
I know exactly what Belfiore expects of me when a client makes contact				
I can quote a price without checking with somebody				
I can answer most treatment questions a client asks				
I am confident with a client who is angry				
I know what to say when somebody is unhappy with her result				
I know who to escalate to, and how fast				
I know which clients are due back and when				
I know what I am allowed to decide on my own				

PART THREE. THE DESK REFERENCE

Keep this part

The two days are over. This is what you use on an ordinary Tuesday.

Everything here is a consolidation of the sessions, with the teaching taken out and only the usable part left. Leave this section open at the desk. If it is in a drawer it may as well not exist.

What to do when

The situation	What you do	Where it is
An Instagram message comes in at 11pm	Acknowledge tonight, real reply by the morning standard	Session 3
She asks the price on the first message	Give it, attach it to what she gets, move to the next step	Session 3
She asks for a discount	Prices are the same for everybody. Restate value, never concede at the desk	Session 3
She asks a clinical question you cannot answer	Do not guess. Commit to a time and come back by it	Session 3
She will not commit to a date	Offer two specific times, not an open question	Session 3
A walk-in arrives while you are on the phone	Acknowledge within five seconds, even mid-call	Session 4
The doctor is running late	Tell her yourself, with a new time, before she asks	Session 4
Two clients in the waiting room know each other	Notice nothing, introduce nobody, acknowledge nothing	Session 4 and 5
Somebody asks whether a named person is a client	The same answer for everybody. Confirm nothing, warmly	Session 5
A colleague asks for a client photograph	Check who is allowed to receive it before anything moves	Session 5
She asks for a photograph to be taken down	It comes down. Consent can be withdrawn at any time	Session 5
A client is angry about a wait or a charge	Acknowledge the specific thing before any explanation	Session 6
A client is nervous and keeps asking the same question	Slow down. Information, not reassurance. Consultation is reversible	Session 6
A client expects to be an exception	Recognise her, hold the boundary warmly, offer something real	Session 6
A client says she will post about us	Reply once publicly, fast, named, move it private, escalate today	Session 6 and 7

The situation	What you do	Where it is
A client is unhappy with her result	Hear, Hold, Hand over, Record. Never assess, never concede	Session 7
A client calls on a Saturday with swelling	Escalate per the map immediately. Do not triage it yourself	Session 7
A complaint arrives and you are about to go off shift	Log it before you leave. Never hand it over verbally only	Session 7
A treated client has not been contacted	24 hours, from a named person, two sentences	Session 8
She replies "I am not sure it has worked"	Treat as an early result complaint. Escalate, do not reassure	Session 7 and 8
She says she is delighted	This is the moment to ask for a review, and only now	Session 8
A client does not show up	Call the same day, warmly. The embarrassment is what loses her	Session 9
A client leaves without booking again	She goes on the recall list today, contacted within seven days	Session 9
Nobody has seen a client in a year	Reactivation call from a named person. No offer, no campaign	Session 9
You do not know whether you are allowed to decide something	Say so, name a time, come back by it. Then get it added to the standard	Session 2

The Belfiore service standard, one page

Copy the agreed version here once Dr Rapu has signed it. This page is the one that settles arguments.

Area	Our standard
Instagram and WhatsApp, opening hours	
Missed calls	
Email	
Out of hours	
Who owns an enquiry	
What is recorded before an enquiry closes	
Giving a price	
Waiting	
Logging a complaint	
Result complaint reaches a clinician within	
Aftercare contact	
Next appointment	

Signed: _____ Date: _____

The escalation map

Situation	Who	How	How fast
Unhappy with a result, no symptoms			
Pain, swelling or a possible complication			
Something she says is an emergency			
Complaint about a member of staff			
A billing dispute			
A public post or review			
A request for a refund			
A lawyer or a regulator makes contact			
After hours, the number to call is: _____			

The phrase bank

Situation	Say this	Not this
She is upset	"That should not have happened"	"Unfortunately our policy is"
You do not know	"I would rather get you the right answer than guess"	"I think so"
You need time	"Let me look into it and come back to you by four"	"I will get back to you"
She wants a discount	"Our prices are the same for everybody"	"Let me see what I can do"
She asks a clinical question	"That is one for the doctor, and I will get it to her today"	"Usually it is fine"
She is waiting	"You have been waiting a while. Here is where we are"	"It should not be long now"
She threatens to post	"I would like to sort this out for you properly"	"That will not be necessary"
She is nervous	"I would rather you asked me ten questions than one"	"Everybody says that"
She is unhappy with a result	"Thank you for telling me. I want the doctor to look at this properly"	"It looks fine to me"
Somebody asks if she is a client	"I am not able to confirm whether anybody is a client here"	"Let me check the diary"
She did not show up	"I wanted to make sure everything is alright"	"You missed your appointment"
Closing a booking	"Shall I put you in for the 14th so you get the time you want?"	"Let me know what works"

The message set

Fill in your agreed wording once, then use it. Consistency is what makes it feel like a practice rather than three people improvising.

First reply to a new enquiry

Price reply

Booking confirmation, with preparation notes and the deposit position

72-hour reminder

24-hour reminder

24-hour aftercare check-in

Seven-day check-in

Review request

No-show call opening

Reactivation, six months quiet

The recall calendar

Treatment line	Recall interval	Who contacts her	What is offered
Botulinum toxin			
Dermal fillers			
Skin treatments and medical facials			
Chemical peels			
Microneedling			
Laser hair removal			
Laser skin and pigmentation			
Body treatments			
Hair			
Wellness and IV			
Post-treatment review appointments			

The recall list is worked every _____ by _____

The weekly numbers sheet

Photocopy this page. One line a week, one person, about twenty minutes.

Week ending	Median first response	Enquiries to consults	Consults to treatments	Rebook at checkout	No-shows

Also watch: complaints closed within 72 hours, and dormant clients reactivated this month.

The weekly huddle card

Fifteen minutes. Same day, same time, standing up.

- The five numbers. Up or down, no discussion yet
- One thing that went well, named and credited
- One thing that went wrong, and what we change. Never who is at fault
- Anything coming this week
- One commitment each

Notes

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